

01-mri-lumbar-spine-policy

Payer Prior Authorization Policy: MRI Lumbar Spine (Without Contrast)

CPT Code: 72148 **Applicable ICD-10 codes:** M54.5 (Low back pain), M51.26 (Other intervertebral disc displacement, lumbar region), M54.16 (Radiculopathy, lumbar region)

Policy Number: RAD-LSP-014 **Effective Date:** 2026-01-01

Purpose

This policy defines the clinical criteria required for prior authorization of MRI of the lumbar spine without contrast in the outpatient setting for adult members with low back pain and/or suspected lumbar radiculopathy.

Required Supporting Documentation

A request is considered complete only when ALL of the following are submitted, unless an explicit exception below applies:

1. **Conservative therapy documentation** — Clinical notes documenting at least 6 consecutive weeks of conservative treatment (physical therapy, chiropractic care, or a structured home exercise program supervised by a licensed provider) within the 6 months immediately preceding the MRI request, with dated visit notes showing the specific modalities used.
2. **Ordering physician statement of medical necessity** — A signed statement from the ordering physician (not a physician assistant or nurse practitioner co-signature alone) explaining why imaging is required at this time and how it will change management.
3. **Neurological examination findings** — For any request coded with a radiculopathy diagnosis (M54.16 or similar), documented neurological exam findings: motor strength testing by myotome, sensory exam by dermatome, deep tendon reflexes, and straight-leg-raise test result (positive/negative/degrees).
4. **Prior conventional imaging** — A lumbar spine X-ray (2-view minimum) or CT performed within the 12 months preceding the MRI request, per step-therapy imaging policy, UNLESS a red-flag exception (item 6) applies.
5. **Red-flag screening documentation** — Explicit clinical documentation that the following have been considered and are absent: cauda equina syndrome (saddle anesthesia, bowel/bladder dysfunction), suspected vertebral fracture, spinal infection, and malignancy with spinal metastasis. A simple absence of mention does NOT satisfy this requirement — the note must affirmatively address red flags.
6. **Conservative-therapy bypass exception** — If conservative therapy was not completed, the request MUST instead include documentation of one of the following bypass criteria: progressive motor deficit, new bowel/bladder dysfunction, suspected cauda equina syndrome, or suspected spinal infection/fracture/malignancy. When this exception is used, item 1 is not required, but items 4 and 5 remain required.

7. **Prior imaging results (optional)** — Any prior lumbar MRI, CT, or X-ray reports relevant to the current complaint, if available, to support comparison.

Exclusions

- Requests for MRI as a routine screening exam without a qualifying diagnosis code are not authorized under this policy.
- Repeat MRI within 90 days of a prior MRI for the same complaint requires documentation of significant clinical change or a new red-flag finding.